

19/01/2022

Dr Anita Arora
Obstetric Department
Melbourne Maternity Centre
East Melbourne 3007

Re: Mrs. Kathy Jonnas Shawn, 26 years old
Dear Dr Arora,

I am writing to refer Mrs Kathy Jonnas Shawn, a 20th week G2P0 pregnant lady, who is demonstrating signs and symptoms suggestive of pre-eclampsia, for your urgent care and assessment.

Regarding her history, Kathy suffers from intermittent asthma since 2014, underwent an appendectomy ten years ago, and had pelvic inflammatory disease in 2013. Furthermore, her first pregnancy ended in miscarriage at 12 weeks in 2019 due to pre-eclampsia, and she was diagnosed with antiphospholipid syndrome three months later. Please note, she is allergic to ~~dusts, pollen and~~ penicillin, she smokes ten cigarettes per day, and she consumes alcohol and crack occasionally. Her family history is positive for diabetes, hypertension and pancreatic cancer. Lastly, she was prescribed Aspirin 100 mg per day since her fifth week of pregnancy; however, she was not consistent with her follow-up appointments.

Today, Kathy presented with dizziness, headache, nausea, vomiting, blurred vision, abdominal pain and vaginal spotting for 24 hours. On examination, her blood pressure was 152/90, pulse was 90 and respiratory rate was 22. Moreover, bilateral pitting pedal edema, peri-orbital edema, facial puffiness, tenderness in the right upper abdomen and confusion were noted. Finally, fetal position was oblique with faint heart sounds.

Based on the above data, urgent referral of Kathy is needed to control of her blood pressure and close monitoring to prevent development of eclampsia.

Should you have any queries, please do not hesitate to contact me.

Yours faithfully,
Doctor

Commented [ESL1]: (Mention the patient's full name in the reference line and introduction paragraph.)

Commented [ESL2]: urgent management

Commented [ESL3]: (Sequence the information logically based on importance.)

(Miscarriage and then the remaining medical history)
(Try to write social and family history in this case in a separate paragraph to give importance to medical information and for clarity.)

Commented [ESL4]: Mrs Shawn
(Use last name/surname with appropriate title when referring to an adult in body paragraphs.)

Commented [ESL5]: (The following connectors/linking words are not accepted in OET writing: so, because, moreover, meanwhile, besides, furthermore, thus, hence, thereafter, apart from this/that and later on)

<https://edubenchmark.com/blog/list-of-linking-connectors-to-avoid-in-oet-letter-writing/>

Commented [ESL6]: and

Commented [ESL7]: Please note

Commented [ESL8]: a
Brand: Upper Case (first letter)
Generic: Lower Case (first letter)

<https://edubenchmark.com/blog/oet-writing-tips-capital-letters/>

Commented [ESL9]: during

Commented [ESL10]: (This paragraph discussing recent information can be written after introduction as Body Paragraph 1 as it is an urgent referral.)

Commented [ESL11]: Additionally

Commented [ESL12]: On obstetric examination

Commented [ESL13]: Mrs Shawn is urgently referred to you for optimal
(Involve the addressee while mentioning the expected care/action mentioned.)

Commented [ESL14]: sincerely
(The name of the recipient is given.)

Report	
Word Length	210
Comments	The purpose is stated, but it is not developed clearly in the introduction. The letter tries to include relevant case notes, but it includes irrelevant information/details also. The content is not organized based on the letter type (urgent referral). The patient is not referred to properly in places. In addition, there are inaccuracies related to capitalization, vocabulary, punctuations, basic grammar and sentence structure. The complimentary close used at the end is not appropriate. Overall, the letter needs improvement.
Estimated Grade	C+
Estimated Score	320/500
Advice	1. Develop the purpose of the document clearly in the introduction. 2. Always read and select case notes carefully. 3. Refer to the patient appropriately in the letter. 4. Avoid basic grammar errors. 5. Punctuate the content appropriately. 6. Pay attention to capitalization. 7. Improve choice of words. 8. Improve the structure of sentences. 9. Avoid using outdated/informal/academic connectors.

	10. For urgent referrals, give priority to the most recent symptoms and needs. 11. Improve paragraphing. 12. Try to finish the letter in 200 words by writing information in brief wherever possible. Watch this video on word length: https://www.youtube.com/watch?v=AyaIEjPv6Ag 13. Use appropriate salutations in the letter. 14. Always proofread the letter after finishing it.
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